

University Cancer Institute - Leukemia Clinic
PROGRESS NOTE

DATE OF VISIT: March 20, 2024

SUBJECTIVE:

Ms. Howard is a 61-year-old female with intermediate-risk AML, diagnosed on June 14, 2023 and started on HMA/BCL2-targeting treatment a week later. She is feeling very well. She completed her 9th cycle of Venetoclax + Azacitidine last week. She reports good energy levels, allowing her to take daily walks and engage in her hobbies. She has no fever, chills, or new complaints. She notes mild, chronic constipation, which she manages effectively with Miralax. Her adherence to oral Venetoclax has been 100%.

ASSESSMENT:

Ms. Howard has had an excellent and sustained response to her first-line therapy with Venetoclax + Azacitidine for her intermediate-risk (cytogenetically and molecularly normal) AML. Her blood counts have been stable for the past 6 months, and her bone marrow biopsy from two months ago confirmed a continued MRD-negative complete remission.

The primary topic of discussion today was the optimal duration of therapy. I explained to Ms. Howard that this is an area of active investigation in the hematology community. Unlike traditional chemotherapy, which has a defined endpoint, the ideal length of treatment with Venetoclax + Azacitidine for patients in deep remission is not yet established.

We discussed the risks and benefits of continuing versus stopping therapy:

- **Continuing Therapy:**
 - **Benefit:** May prolong remission and prevent relapse.
 - **Risks:** Continued risk of myelosuppression and infection, ongoing need for clinic visits and injections, financial toxicity, and "pill fatigue."
- **Stopping Therapy:**

- **Benefit:** Freedom from active treatment, recovery of normal blood counts, improved quality of life.
- **Risk:** Potential for the leukemia to relapse. The risk is likely lower given her MRD-negative status but is not zero.

PLAN:

After a shared decision-making discussion, Ms. Howard expressed a desire to continue therapy for now, as she is tolerating it well and prioritizes doing everything possible to prevent relapse. We agreed on the following plan:

1. **Continue Venetoclax + Azacitidine.** We will aim for a total of at least 18 to 24 cycles of therapy.
2. **Monitoring:** We will continue monthly blood counts. We will repeat her bone marrow biopsy with MRD assessment every 6 months.
3. **Re-evaluation:** If she remains in an MRD-negative remission at the 18-month mark, we will have another in-depth discussion about the possibility of discontinuing treatment at that time.
4. She will receive her 10th cycle of Azacitidine injections starting tomorrow. A prescription for her next cycle of Venetoclax was sent to her specialty pharmacy.

PATIENT: HOWARD, Alice

MRN: AML068

DOB: August 7, 1962